

Psychic room to breathe. Themes emerging within a staff Balint group on an eating disorder inpatient unit

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Anorexia nervosa is the psychiatric disorder with the highest mortality rate. Anorexic patients admitted to inpatient eating disorder units are often critically unwell in both their physical and mental health. Underpinning working with these patients is a real feeling that they are close to death. Staff, and particularly nurses, in eating disorder units may find working with anorexic patients extremely challenging. It has been documented that working with such patients can give rise to strong and difficult to process countertransference feelings and reactions. The authors of this paper introduced a Balint group for the staff on an eating disorder inpatient ward to provide a space for the team to reflect on the work. This paper reports on the experience of running this Balint group and describes the themes that gradually emerged. These are then discussed and related to the nature of the disorder and the particular challenges of working with this inpatient population.

Introduction

Anorexia nervosa is the psychiatric disorder with the highest mortality rate, carrying an almost six-fold increase in the risk of premature death (Arcelus et al., 2011). Admission to an inpatient unit is rare and is only for the most unwell, those at risk of medical complications, or suicide. The usual body mass index (BMI) of patients when admitted is very low and they can be close to death for prolonged periods during their admission. The impact on staff working daily with this perilously ill patient population has not been thoroughly examined, though it has been documented that working with patients with anorexia can give rise to strong countertransference feelings (Hughes, 1997; Marcinko, 2015; Satir et al., 2009).

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1

This paper reports on the experience of running this Balint group and describes the themes that gradually emerged. These are then discussed and related to the nature of the disorder and the particular challenges of working with this inpatient population. The group material has been anonymised and no identifying information is included in this paper. To our knowledge this is the first paper reporting on a Balint group for staff in an eating disorder setting.

Reflective practice and Balint groups

Reflective practice provides a space for teams to express, consider and process the feelings engendered by their clinical work. Provided regularly, this space allows for deeper exploration and insight into the work and the alleviation of professional distress. There are diverse models of reflective practice, from the psychoanalytically informed, such as Balint groups, where the reflection is fuelled by the free associative discussion of the group, to the cognitive, such as the Gibbs Reflective Cycle (Gibbs, 1988). Other forms of reflective practice are generally less overtly focused on the clinical dyad and countertransference compared to Balint and Balint-type groups.

Balint groups were first set up in the UK in the late 1950s by Michael and Enid Balint, initially as group seminars for General Practitioners (Balint, 1957). They offer a structured form of psychodynamic reflective practice, which allows members with little psychotherapy or group experience, to explore the emotional aspects of the patient-clinician relationship in a contained manner. The group meets for an hour and there are one or two group leaders. The group members are offered the opportunity to present a case. After an informal presentation, which concentrates on the presenter's emotions and concerns about the case as much as the clinical management, the presenter takes a listening position while the group reflects on the patient-clinician relationship using their own free associations. This process allows processing of the members' emotions towards the patient and the revealing of unconscious processes that may be present, thus potentially allowing the clinician and the group a new perspective regarding the patient. A number of studies show that Balint and Balint-type case discussion groups enhance psychological skills, empathy, attitudes towards patients and colleagues and job satisfaction, and reduce burnout (Ghetti et al., 2009; Kjeldmand & Holmström, 2008; Santos et al., 2018; Sekeres et al., 2003; Turner & Malm, 2004).

The setting

The Eating Disorders Service is a large specialist service, with an inpatient unit. The unit treats some very unwell, principally female, anorexic patients. For these patients, the disordered eating behaviours tend to be deeply ingrained and have a profound impact on any time when there might be an expectation of eating. Some patients are detained under the Mental Health Act, and a small minority may require feeding by naso-gastric (NGT) or percutaneous gastric (PEG) tube.

2

The multi-disciplinary team consists of psychiatrists, psychologists, psychotherapists, nursing staff (mental health nurses and healthcare assistants), occupational therapists, dieticians, and physiotherapists. All staff receive regular clinical supervision. The therapeutic program is structured and intensive, and includes supervised meals, various group activities, individual and family therapy sessions, and medical reviews. Weight is regularly measured and is used as a concrete indicator of progress in treatment. There is a weekly ward round where patient care is discussed, and different members of staff bring their own picture and understanding of the patient.

The anorexic's internal world

Anorexia has been understood in psychoanalytic literature as an inability to negotiate the oedipal stage of development and a failure in separation. In the anorexic's internal world, the parents have not been internalised as a lively connected sexual-creative couple. They remain psychically split. The anorexic is locked in a pre-oedipal fusion with an idealised mother, whereas the father is experienced as absent and insignificant. The lack of linking between the parental objects makes the psychic linking needed for the capacity to think deficient and the resultant internal world is barren. Thinking becomes concretised and there is a limited capacity to symbolise, process loss, and learn from experience. The patient remains stuck. Any move towards separation gives rise to intolerable persecutory anxiety. There is thus a regressive move away from life towards the anorexic deadly state of absolute control of the internal world and the denial of neediness, as a means for survival. Due to the patient's limited capacity for symbolisation, the maternal function becomes concretely associated with food and is rejected because being nourished by another is perceived as intrusive. The aim of this omnipotent attack on hunger is to provide some concrete separation, but to prevent any real linking. The

patient unconsciously intends to stop time and avoid her development into a sexual-desirous being. An alternative is sought which consists of a controlled merging with an idealised internal featureless (maternal) object, which has no qualities of life that might be wanted or needed (Birksted-Breen, 1989; Lawrence, 2001, 2002, 2008).

Challenges of working with anorexic patients in an inpatient setting

Anorexic patients admitted to inpatient eating disorder units are often critically unwell in both their physical and mental health. Underpinning the relationships within the team and with the patients on the unit is the real feeling that these patients are close to death. This is a preoccupation for both the patients (some of whom are not able to resist the drive to maintain themselves as close to death as possible) and for staff members who often focus their work on the practicalities of trying to keep the patients alive. The primary focus of treatment is frequently on weight restoration and medical stabilisation. Staff working with eating

3

disordered patients may adopt a concrete view of the patient's condition (over-focus on weight gain, calories, exercise, and practical issues in the unit), which can be an obstacle to thinking about the underlying psychological processes driving the condition. As much as this concrete view may at times be necessary, the professional enactment of physical action on the body mirrors the limited capacity for symbolisation in anorexia.

Effect on staff dynamics

The above picture is complicated by transference processes that occur in eating disorder units (Golan et al., 2009). Different members of staff may be at the receiving end of different projections and transference phantasies. In addition, a multi-disciplinary team may be undergoing 'split transferences', whereby staff members may be separated into all-good and all-bad part-objects (Marsden & Knight-Evans, 2009). The splitting of the external team mirrors the split between the patient's internal objects. The nursing team working closely with the patient may be subjected to intense maternal anxiety and the medical team may often become marginalised and detached (Lawrence, 2002). Such team functioning was described by Tom Main (1957) in his seminal paper *The Ailment*. Main was a psychiatrist who trained as a psychoanalyst under Michael Balint.

He led a group on an in-patient ward who worked with challenging 'special' patients. He noted that there appeared to be splitting between the nurses, who attended the group, and doctors, who did not. He was of the opinion that the doctors were uneasy exposing their feelings and did not have the same need for the group as the nurses. In addition, he suggested that the patients exacerbated pre-existing hidden tensions and divisions by primitive projection of their infant needs. Such transference projections may give rise to strong countertransference feelings within the team and disagreement between staff members (Davey et al., 2014) who compete to satisfy these needs and be the 'perfect' mother. Golan et al. (2009) point out how splitting can create a culture of blame, and staff members may feel 'confused, insecure, demeaned, humiliated, and attacked'. Countertransference reactions of rejection or identification may lead to dynamics of under management or over management (Goodsitt, 1997; Golan et al., 2009), and staff may risk acting in a sadistic way (Marsden & Knight-Evans, 2009), whilst others may collude with the patient in order to attract their love and admiration (Golan et al., 2009). Parallel processes spanning the patient's family, the unit and the organisation may reproduce power struggles at different levels or give rise to parallel re-enactments that create conflict and may compromise the therapeutic and containing function of the unit (Golan et al., 2009; Grey & Fiscalini, 1987).

Effect on staff-patient dynamics

For both the patients admitted to an eating disorder unit and for staff that work there, it may feel that the work on the unit has the character of a conflict, centred on meal

4

times. The question asked by the team would sometimes lean towards: 'how can I persuade this person to eat more and exercise less?' Eating disorder units have been described as 'battlegrounds' in previous studies. A notion of 'us against them' can be prevalent (Long et al., 2012; Ryan et al., 2006; Wright, 2010). Staff have reported feeling like prison officers punishing people (Long et al., 2012) and anorexic patients have also described their experience in prison terms (Ramjan & Gill, 2012).

Staff, and in particular nurses who are on the front line of the work and subjected to the most intense transference experience, may find working with anorexic patients extremely challenging. In a study by Strober (2004), staff commented that 'compassion for the sufferer can quickly reach a limit'. In another study by Walker and Lloyd (2011), professionals reported a difficulty in

comprehending reasons for the eating disorder and feelings of reluctance in working with the patients. They felt that they had less knowledge of eating and eating disorders than the patients themselves and also that the patients did not want to receive treatment. In addition, it has been shown that nurses may find these patients 'deceitful', 'untruthful' and 'untrustworthy', and consider it difficult to build up a 'trusting relationship' (Ramjan, 2004; Snell et al., 2010).

Preventing burnout

It is well recognised that managing difficult emotions and countertransference reactions improves patient care and reduces staff burnout. Burnout is used in this context as a depressive reaction to the prolonged experience of clinical hopelessness derived from the loss of professional ideals. Faced with the anorexic's attack on liveliness and desire, clinicians may come up against a failure of reparative processes that they themselves need in order to restore their own internal objects, and they may suffer a related loss of self-esteem. Symptoms include: exhaustion, depersonalisation, detachment, irritability and indifference to distress and pain. Professionals that are 'burnt out' can appear functional, yet the pleasure, joy and meaning they previously gained from their work is now limited or absent (Maslach & Leiter, 2016).

Golan et al. (2009) suggest that good teamwork requires the establishment of a consensus in regard to treatment and that sharing is very important in counteracting acting out. Working through negative emotions, acknowledging ambivalence and seeking to understand the patient from a developmental perspective can all ameliorate strong countertransference reactions. Consistent supervision, including group supervision (Davey et al., 2014), and work discussion groups (Golan et al., 2009) have all been identified as of paramount importance. Essentially, countertransference reactions and enactments are inevitable, but if they are identified, discussed and understood, they can offer insight into the patient's struggles and can contribute to better care (Marsden & Knight-Evans, 2009).

The Group

A Balint group was introduced and ran on a fortnightly basis, in order to increase the capacity for team reflection on the emotional aspects of staff interaction with patients, including countertransference feelings, outside the more practical side of their clinical commitments. The initiative was taken by

two trainee psychiatrists in the unit, following discussions with other members of staff. The need emerged at a time when staff were caring for some particularly complex patients and it was observed that splitting processes were strongly operative in the team. A staff survey showed that the patient's 'resistance to change' and 'ambivalence' were the most challenging aspects of the clinical interaction for most staff, with nursing staff in addition pointing out many patients' 'over-attachment' to them.

All clinical staff working in the inpatient unit were invited to attend the group on a voluntary basis. The group was facilitated by an external group leader who was an experienced specialty trainee in medical psychotherapy, supervised by a consultant medical psychotherapist. The decision to have one group leader appeared to be justified logically, both culturally and practically. The other Balint groups in the Mental Health Trust ran with one group leader. This also reduced the demand on the resource of the psychotherapy department, which offered this as an extra commitment. However, the lack of reflection about this could have been related to the inherent difficulty in creating a parental couple in this environment. The group was multi-professional and differed from a typical Balint group in that the members were presenting patients with the same psychopathology, namely an eating disorder.

The group ran for one hour and an individual case would be discussed. The group leader started each group with the question 'who has a case?' A group member would volunteer to present, largely nominated by unconscious processes. They then had up to 10–15 minutes to talk about the patient they had on their mind. If necessary, the group leader would encourage the focus on emotional responses. Clarifying questions from the group were kept to a minimum to ensure the time taken for association to the material was maximised. The presenter sat back and listened to the free associative discussion of the group. After around 20 minutes they re-joined the discourse and fed back their thoughts on the discussion. Process notes were taken and discussed in supervision where gradually over time themes that emerged were identified. The group was initially offered for one year however its importance was recognised by both staff and the group leader, therefore senior support was given for it to continue.

The group met in a large meeting room on the inpatient unit. Attendance varied from 3 to 10 members of the team, with an average of 5. Psychotherapists, a psychologist and nursing staff were the main attenders. Psychiatrists initially attended but their attendance gradually dropped off. Other clinicians were present when possible. The group eventually self-selected those who wanted to attend. The nursing staff appeared to consist of either

very junior or the most senior staff, and they tended to be there either as a group or not at all. The psychotherapists and the

6

psychologist were the most regular attenders and the ones that provided a certain sense of continuity. Some group members struggled to arrive on time, with psychotherapists and the psychologist maintaining the time boundary. The getting together of the team was clearly valued, but the struggle to attend as a whole appeared to mirror the patient's internal situation.

Themes

Hard to psychically feed

This was a Balint group where some members found it difficult to feel at ease and allow themselves to have a nurturing experience. Williams (1997) discusses how 'good feeders', patients 'craving for insight', are experienced by those caring for them as valuing of what is being offered and grateful, while 'poor feeders' can provoke feelings of helplessness. Shaped by the experience of caring for the anorexic patients in the unit, some of the members of the group found it difficult to be 'good feeders'.

The absent patient

Something that became clear on listening to case presentations was that it was sometimes difficult for staff to discuss the personalities and the personal and relational background of the patients. It was as if the patient primarily existed in the context of their admission or in the context of the illness. Clinicians regularly presented the patient as though the true person was hidden or absent, although there were times when some could fantasize about who this patient might be if they were well: 'I can imagine her as a dancer or going to university'. It was difficult for the group leader to grasp a mental image of individual patients discussed each week, but rather it felt as if the same patient was presented week after week: a patient who was partially missing or invisible, itself perhaps a manifestation of their barren internal world.

When a patient was presented and discussed, they were recognised by many members of the group and it would become apparent that different aspects of the patient were experienced by professional groups. Splits between professional groups sometimes appeared when staff talked about the same

patient and this could at times create tensions and misunderstandings. Some members felt that recorded notes about the patient focused more on the biological and the body, and described the patient in numbers (such as calories intake, blood results, allocated leave, or exercise time). It was noted by the group that this seemed to parallel how the patients described themselves.

Anger, revulsion and disgust

Certain negative feelings towards patients, such as anger, revulsion and disgust, which emerged on different occasions, were difficult to express and be thought

7

about. Many group members felt that they had to preserve positivity in their relationship with patients, angry feelings were difficult to admit to and were associated with unsettling feelings of guilt. Even though disclosing these feelings presented a challenge, there was often a sense of relief in the group when they were expressed. Clinicians' anger ranged from irritability to frustration with a perceived lack of progress. Anger was also sometimes expressed in response to verbal assaults from patients. Other emotions that were very difficult for members of the group to admit were revulsion and disgust. These feelings came into the group occasionally, for example, when discussing a patient who had smeared faeces or resulting from a particularly difficult interaction where a patient had to be physically restrained or struggled with PEG feeding. Disgust was also identified as the feeling members of the group had about themselves when carrying out physical interventions or feeling as if they had intruded on a patient's privacy.

Controlling/controlled patients and controlling/controlled clinicians

Control was an important topic for the group. It was brought up numerous times, initially as the countertransference difficulty arising from feeling controlled by the patient. This seemed to manifest as refusal and a lack of cooperation with the mutually agreed care plans. An intense degree of negotiation was required to make a small change to a plan. However, this theme of control applied both ways, as staff described the need to implement a treatment plan with the patient, when that patient was focused on starvation. There was also sometimes a sense of concern if patients presented with symptoms of a comorbid condition, such as psychosis or borderline personality disorder. Some patients who were discussed in this context were described as

presenting the ward environment with more confusion and a departure from the usual ward routines.

The clinician under fire

The language of the group was on some occasions one of 'war' or 'battle'. This theme was also noted in the general literature, as documented above. In particular, nursing staff talked about having to 'put on armour' to face certain patients and 'battle' to ensure care plans were adhered to, feeling many times verbally assaulted by patients. The team encouraged basic life support, fighting against the patient's efforts to be consumed by something deadly. They aimed to keep them alive until they were at a point they could do this themselves. It was accepted that once a clinician felt 'on the same side as' the patient, the patient was often well on their way to recovery. In this way the intense battle between the life and death drives of the patient themselves was projected and enacted in the external ward environment (Lawrence, 2002).

On two occasions, patients were transferred to other units and the group reflected on what this transfer might mean. It seemed to represent two different

8

possibilities: (1) new hope of change, with fresh staff, or (2) hopelessness about the future. The group communicated despondency and a feeling of failure at not being able to treat the two patients successfully. There was a sense that in these cases the battle between life and death had not been won. This perceived failure meant the team had to face the power of the anti-life forces they were confronting daily. The relief at being released from this painful and apparently fruitless battle to treat a patient who was resistant to all their efforts was difficult to acknowledge. Both hopelessness and relief were associated with feelings of guilt in the group, which were experienced as unsettling.

Facing death

It took a long time before death was actually named in the group, but this theme stayed with the group over several months in different forms and allowed for more overt work on the daily struggle between life and death to take place. The question the group faced was: why was it hard to recruit staff and especially nurses into eating disorder units? The answer the group arrived at was that these patients were so close to death that it could at times make caring for them very stressful.

The nurses expressed a feeling that other mental health teams did not appreciate how mentally and physically unwell these patients are. The psychiatrists in the group agreed and described how the added complexity of managing the physical care of these patients was anxiety provoking, a process that sometimes took their minds away from thinking about the patient in a more psychological way. A group member said: 'every time I take her blood pressure or touch her, I hold my breath, as if she might die from the procedure'.

It was initially unspoken, but eventually a comment came into the group that it felt taboo for a patient to die on the ward. Deaths generally occurred on medical wards and so it was hard for them to imagine a 'good' or 'appropriate' death on an eating disorders unit. This discussion brought up a theme of whether the concept of palliative care was an appropriate one for those with anorexia nervosa, but eventually the group rejected this: 'One could never tell when they might get better'. But rejecting the option of palliative care left the group wondering how it was possible to maintain hope for these patients. The other question the group was faced with was whether any of the deaths from anorexia nervosa could be considered as a form of suicide or deliberate self-harm. Everyone in the group was in agreement that they were treating a mental illness and that self-destructive actions were to be attributed to 'anorexia'. On the other hand, patients with other diagnoses who carried out suicidal or para-suicidal acts that placed their lives at risk were thought to be more responsible for their actions, which could be harder for the ward to tolerate. Overall, it was difficult for the group to stay with the thoughts and feelings about an actual death happening on the unit, despite its daily ever-presence.

9

Death was the theme that most preoccupied the group leader who felt in almost constant contact with the fear of death in working with this fragile patient group. The group and the group leader were aware that there were times when patients would repeatedly need urgent medical attention to stay alive, requiring almost constant monitoring. Some patients would have periods of being transferred almost daily to the local Accident and Emergency department, only to be returned to the ward the day after admission. The group leader was surprised, given how taken up they were with the deadly forces of anorexia, how long it took for death to emerge as a theme in the group. There were times when it felt that to discuss the fear of death may concretely bring death to the group itself. The initial introduction of this topic explicitly in the group gave rise to paranoid anxiety that took time to dissipate.

Discussion

The group leader became aware early on of the tremendous impact on every individual clinician, professional group, and the team as a whole, of the constant daily contact with the anti-life psychic forces of anorexia nervosa. This was experienced in numerous forms that included: controlling behaviours, rigid routines, stubbornness, passive aggression, resistance to change, verbal assaults, lack of gratitude, hyper-rationality, concrete focus on numbers and actions, inability to engage in a mutually nurturing relationship, starvation and the very real possibility of actual death. This close proximity to death in this young patient group can create high levels of anxiety in those looking after them, who are made to feel that they are high wire artists in the tightrope walk of the anorexic patient.

The operation of certain defensive processes within the team became apparent in the dynamics of the group and it is worth highlighting how these seem to reflect the defences and psychological profile of the anorexic patients. The concrete focus on biology and the mechanics of treatment (weight, calories, mealtimes, exercise, leave etc.) functions as a safe retreat, especially when staff members are faced with overwhelming feelings – be it anxiety, anger, helplessness or terror, which may result in undermining a psychological understanding and engagement with the patient's internal world. When medical stabilisation of the patient's health is the priority in the unit, one can see how this can present as an obvious defence.

There can be an attempt by staff to find an ally in the sane part of the patient's personality by objectification of anorexia as an illness separate from the patient's personality – 'it is not you, it is the anorexia', but at the same time this can obstruct an understanding that, as Hughes (1997) writes, 'anorexia is a solution to another more alarming problem'. The patient's individual personality, their relational background, and their conflicts can get lost. It is as if holding in mind a whole person in such a state of methodical self-destruction is emotionally unbearable. In a way, on many occasions the anorexic's desire to be

invisible may be materialised in the way the staff allow themselves to see – or at times not to see – these patients.

The proximity to death and the deadly grip of anorexia was a subtext in most discussions. When this was eventually named, the group was able to start thinking about how their anxiety about this made their work extremely stressful

at times. However, the theme of death remained for most part a taboo, as if the group felt overwhelmed by unbearable anxiety in the prospect of thinking through this. In addition, a denial of death was manifest when the group discussed that death should be happening somewhere else and not in the unit, as well as in the group eventually rejecting the idea of palliative care.

What was striking was the language of 'battle' used by some group members, especially nursing staff, and how consistent this was with previous research findings. There was a sense that the 'battle' professionals found themselves engaged in, is a battle between life and death, with staff holding the side of life projected into them by an ambivalent patient, who can then align themselves with the deadly forces.

The group generally found it difficult and unsettling to discuss counter-transference feelings and reactions. However, some strong feelings made it to the group and it was accepted that being able to name them, share and discuss them, was a relief. Such feelings included hopelessness, guilt, anxiety, irritability, frustration, and anger.

Transference was rarely discussed explicitly, however multiple and split transferences were clearly visible within the group. Unacknowledged anxiety within the team as well as the multi-professional character of the group made a fertile ground for splitting processes to operate. Frontline nursing staff sometimes argued that they are at the forefront of caring for these very unwell patients, experiencing directly all their stubbornness, resistance, aggression, and complaints, whilst other professional groups, such as psychiatrists and psychotherapists, are more protected from continuous direct patient contact, formulating treatment plans without taking into account the difficulties of their practical implementation. The tensions caused by these splitting processes at times made members of the group defensive, so it was difficult to think about how such different experiences are also the result of patients projecting different parts of their minds and their internalized family dynamics to different clinicians and are an example of the attacks on the parental couple which are characteristic of the psychopathology of anorexia nervosa (Lawrence, 2008).

Limitation

It is a limitation of this paper that we did not use a formal research methodology (such as thematic analysis) for identifying themes. Detailed process notes were kept by the group leader and discussed in supervision, where the themes described above were identified and reflected upon. They

were further elaborated and clarified in the discussions of the authorship group.

Conclusion

Balint groups can help identify underlying unconscious processes and defences in clinical teams. This group has been valuable for the team and much appreciated as a reflective space in the unit, despite the challenges and tensions. Members of the group have managed to name, share and reflect upon some very difficult feelings, and this was experienced as a relief and reduced some of the anxiety within the team. The group has also had a positive impact in increasing staff's curiosity about themselves, their patients, and their interest in learning from other professional groups. They at times came to realise that often different aspects of the patients were being experienced by different professionals and only a well-functioning multi-disciplinary team can offer some understanding of such fragmented patients. When feedback was formally sought 6 months after the commencement of the group, the aspect that staff appreciated most was being able to get together and feel like a team, sharing and learning from others. There seemed to be a hunger for creative linking in this environment where both hunger and linking are constantly under attack. Staff believed that the group helped them in their everyday work and contact with patients, strongly expressing a wish for it to continue past its first planned year, which it has.

What we find particularly interesting is that through this process of 'making the unconscious conscious', with its conflicts, difficulties, and blind spots, certain defence mechanisms operating within the team have become apparent and they seem to reflect the anorexic's defences and conflicts.

We summarised above the importance reflective spaces and supervision have in managing difficult emotions and countertransference reactions. Maintaining such thinking spaces can actually help staff gain insight into such processes, thus making them better able to manage their working environment, improve teamwork, and reduce burnout, as well as gain more insight and be able to formulate what might be happening for an individual patient – something that is not always easy.

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